



## Policy

# Patient Discharge and Discharge Planning

## Scope

| Site     | Service/Department/Unit                                   | Disciplines |
|----------|-----------------------------------------------------------|-------------|
| SCGOPHCG | Organisation Wide<br>(excluding the Emergency Department) | All Staff,  |

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## Introduction

The discharge of a patient is the responsibility of all health care professionals involved in the care of the patient. A patient is considered discharged when their episode of care is completed.

Discharge planning should commence prior to admission for elective patients and at the time of admission for non-elective patients. All discharge activities should focus on coordinating services effectively to prevent unnecessary delays and ensure the discharge occurs expediently.

This policy includes discharge of patients from inpatient same-day or multi-day beds.

This policy does not include Discharge against Medical Advice (DAMA). Refer to the [SCGOPHCG Detainment and Discharge Against Medical Advice policy](#).

## Risk Statement

Non-compliance with this policy will:

|                                       |                          |                                   |                          |
|---------------------------------------|--------------------------|-----------------------------------|--------------------------|
| Breach legislative requirements       | <input type="checkbox"/> | Impact on Patient Quality of Care | <input type="checkbox"/> |
| Breach National/State/Hospital Policy | <input type="checkbox"/> | Impact on Patient Safety          | <input type="checkbox"/> |
| Breach professional standards         | <input type="checkbox"/> | Misconduct                        | <input type="checkbox"/> |
| Breach SCGOPHCG Mission & Values      | <input type="checkbox"/> | Staff Safety                      | <input type="checkbox"/> |
| Other:                                | <input type="checkbox"/> |                                   |                          |

## Definitions

|                               |                                                                                                                                                                                                                                   |
|-------------------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>webPAS</b>                 | A state-wide enterprise web-based system used by public health service providers to record their patient visits. The information is broadcast to a wide range of clinical applications to link visits and clinical records        |
| <b>iSOFT Clinical Manager</b> | An electronic patient information system that maintains a repository of clinical information including PathWest Laboratory and Imaging results                                                                                    |
| <b>eReferrals</b>             | An enterprise web-based application designed to manage consults from creation to completion. The solution supports management of medical, allied health and nursing inpatient and outpatient referrals within and across the site |
| <b>NaCS</b>                   | Notification and Clinical Summaries (NaCS) is a web-based application for managing patient discharge summary information and prescriptions                                                                                        |
| <b>EDD</b>                    | The EDD is the date inclusive of a time that the patient is likely to be ready for discharge based on clinical judgment made by the medical, nursing and allied health staff                                                      |
| <b>EBM</b>                    | EBM Enterprise Bed Management (EBM) is the approved bed management application for recording all bed requests                                                                                                                     |

## Policy Principles

Formal discharge is the administrative process, by which a hospital records the cessation of treatment and/or care and/or accommodation of a patient, where the patient:

- Is discharged to private accommodation or other residence (with no intention to return to the hospital within seven days for the continuation of the same treatment)

- Is transferred to another hospital, health service or other external health care accommodation (an exception to this is when there is the intention that the patient will return within seven days for continuation of the same treatment – in which the patient will be placed on leave)
- Leaves against medical advice (see [Detainment and Discharge Against Medical Advice policy](#))
- Fails to return from leave within seven days (discharge date to be entered as the date the patient went on leave)
- Is deceased

## Procedure

Discharge and transfer from SCGOPHCG should occur as soon as the patient no longer requires acute or clinical care at SCGOPHCG, or their care could be better provided outside the hospital.

The authority to discharge a patient from hospital will follow a consultant/team decision, and this process must be:

- Timely
- Clearly documented in the medical record and discharge summary
- Discussed and completed in collaboration with the multidisciplinary team, patient, family and carers.
- Communicated to all health care professionals involved in the care of the patient
- Recorded on webPAS.

Patient discharges and transfers will be planned to ensure that patients have the necessary:

- Information
- Services after discharge
- Medications and/or supplies
- Transportation
- Equipment

Community based programs such as HomeLink, RITH and Silver Chain should be considered for all appropriate patients.

Discharge of SCGH patients must be facilitated through the Transit ward unless clinically inappropriate. Transit ward hours are 24/7 across 7 days..

## 1. Estimated Date of Discharge (EDD)

The EDD is to be recorded on the patient bedside EDD board, the ward journey board, in the patient's medical record and on webPAS:

- On admission for elective (waitlist/non-waitlist patients)
- Within 12 hours of admission for non-elective patients
- Upon transfer between wards/specialties

The EDD will be reviewed:

- When there is a change in the patient's condition
- At the time of daily review.

Changes to the EDD will be:

- Discussed by all health professionals involved in the care of the patient

- Communicated to the patient and their family/carer by staff throughout the patient stay
- Documented on the journey board and the bedside EDD board
- Updated in webPAS in the "Update Condition" section by the ward clerk
- Communicated with rehabilitation and other discharge services if required (see section 5.)

The EDD for patients with complex clinical issues or complex discharge requirements will be reviewed at each multidisciplinary meeting and:

- Communicated to the patient and their family/carer by staff
- Updated in webPAS by the ward clerk
- Communicated with rehabilitation and other discharge services if required (see section 5.)

## **2. Senior Clinical Review**

- All inpatients must be reviewed by a senior clinical decision maker daily.
- All ward rounds must be structured to prioritise review of the highest acuity patients and then those identified for discharge.
- All inpatients who are likely to be discharged from SCGOPHCG the following day must have a criterion for discharge documented in the medical record.
- Patients who remain on the ward on a Friday must have a weekend plan including criteria for discharge documented in the medical record.

## **3. Documentation of the Discharge Plan**

Discharge planning must be clearly documented in the patient's medical record. Details should include:

- Current readiness for discharge
- Events to be achieved or arrangements to be made prior to discharge including dates and times to be completed
- Identification of current or possible barriers to discharge
- Arrangements for discharge (education, home/other preparation, travel arrangements)
- Equipment to be provided/taken by patient

## **4. Requirements for Discharge**

All services assessed as necessary for patient discharge or in the post-discharge period are to be arranged by the multidisciplinary team to ensure that discharge is not delayed. If IV sedation or same day elective anesthetic has been required during the elective procedure the patient must be discharged to the care of a responsible adult.

### **4.1 Allied Health Services**

- Referral should be made as soon as practicable following admission or after a change in the patient's condition to enable timely assessment and arrangements for discharge
- Allied Health should be notified of changes to EDD to ensure all relevant referrals are made and handover of care is completed prior to discharge and equipment can be provided in readiness for discharge

### **4.2 Phlebotomy**

- Blood tests or other specimens required prior to discharge should be ordered on the evening prior to discharge by the medical team

- The request should indicate “urgent” to ensure results are received in a timely manner and discharge is not delayed.

#### **4.3 Patient Transport**

- Transport arrangements should be confirmed at least 24 hours prior to discharge by ward staff.
- Where possible, it is the patient’s responsibility to organise their own transport home.
- Ward staff at SCGH should contact the Discharge Coordinator for patients with complex travel requirements or for inter-hospital travel to rural/remote areas. Further planning should occur when it is expected that a discharge will occur on a weekend or public holiday.
- Refer to [Patient Transport Services](#)

#### **4.4 Results Acknowledgement**

- All clinical investigation results must be reviewed and acknowledged in accordance with the [SCGOPHCG Results Acknowledgement and Communication policy](#)
- The medical team is responsible for:
  - Signing off on all unacknowledged results in iCM at the time of discharge
  - Identifying in the discharge summary any outstanding results that require follow-up, who will undertake the follow-up and when
  - Ensuring processes are in place to electronically sign off unacknowledged results from tests reported to iCM after discharge.

#### **4.5 Discharge Medications**

- The medical team is responsible for writing discharge prescriptions and for consulting with the ward-based pharmacist in advance of discharge to ensure the patient is ready for discharge from a medication safety perspective.
- Prescriptions should be forwarded to the pharmacist by 1600 hours the day prior to discharge where possible, to ensure;
  - timely clinical review of prescriptions by the ward-based pharmacist
  - dispensing by SCGOPHCG Pharmacy or if required facilitating dispensing by patients’ external pharmacy service (e.g., Webster Pack ® discharges)
  - patient and/or carer receive appropriate medication education
- A patient may take their prescription to have dispensed at a Community Pharmacy. The ward-based pharmacist should review the prescription to ensure:
  - Prescriptions are clinically appropriate and legible
  - Prescriber is contactable (name, pager)
  - PBS compliance
  - The items are easily obtainable from community pharmacies, not uncommon or expensive.

Note that for afterhours discharges where there is no clinical pharmacist available at ward level, prescriptions should be reviewed by medical and nursing staff and given to the patient to dispense at a community pharmacy.

##### **4.5.1 Discharge medications for patients being discharged to residential facilities**

- The residential care facility should be contacted prior to patients being discharged (by doctor, ward pharmacist or social worker) to confirm how medications are to be supplied for the patient.
- If the facility uses a designated Community Pharmacy for medication supplies the ward pharmacist will fax or email the prescription and discharge

medication list to the community pharmacy and liaise to supply the medications.

- Original prescriptions and a copy of the discharge medication list are to be sent with the patient to the residential care facility or posted directly to the community pharmacy.
- The discharge prescription(s) and discharge medication summary list should be prepared 24 hours in advance to enable timely review by the pharmacist.

Refer to [SCGH Medication Management Procedure Dose Administration Aids](#) for further information.

#### **4.6 Medical Discharge Summary**

All patients discharged from hospital (alive or deceased) must have a complete and accurate medical discharge summary completed by the medical staff member authorising discharge. The consultant in charge is responsible for ensuring compliance with this.

Discharge summaries should be:

- Commenced as soon as practical following admission using NaCS
- Completed as soon as practical following confirmation of the EDD to prevent delays in discharge
- Completed by 1700hrs the day prior for patients identified as discharging the next morning.
- Printed the morning of discharge and provided to the following recipients:
  - Patient
  - General Practitioner (GP) - faxed/e-mailed within 24 hours of discharge (if it is not electronically transmitted)
  - Consultant in charge
  - Patient medical record

Deceased patients must have a discharge summary completed within 24 hours.

Discharge summary medication lists should be available to be clinically screened by the ward-based pharmacist, to ensure accuracy and completeness, prior to discharge.

For those patients being transferred to OPH an interim transfer document must be produced on NaCS to facilitate a group transfer and relevant information updated in iSoft.

For patients being transferred to another hospital or facility (except Woods Ward) – a discharge summary must be completed and requirements for transfer/discharge letters should be clarified with the facility accepting the patient transfer prior to discharge. Transfers via an ambulance require a transfer/discharge letter. The Medication History and Management Plan (MHMP), if completed, should be photocopied and sent with the patient for inter-hospital transfer.

##### **4.6.1 Exceptions**

Discharge summaries are not required for:

- Patients admitted to SCGOPHCGH for less than 24 hours
- Patients transferred (on leave) to another health care facility with an expected return.
- Same day elective (booked) procedures (e.g. endoscopy), where the operation report provides the necessary clinical details, copies of which should be sent to the GP. Includes elective patients without complications who stay overnight but are discharged <24 hours.



- Day only patients admitted for:
  - Hemodialysis
  - Chemotherapy
  - Radiotherapy
  - Intravenous therapy where no complications arise, no other treatment is provided and the record creation is automated
- Patients with recurring same day care episodes (e.g. same day infusions, dialysis, hyperbaric therapy) receiving a course of treatment of the same condition over weeks or months. A single global discharge summary covering all episodes is sufficient.
- Healthy (unqualified) newborns (babies in their birth episode, with no peri-natal morbidity).
- Statistical discharges for an episode of care change such as from acute care to rehab require a progress (not final) discharge summary.

When a discharge summary is not required, the minimum information recorded on the patient's health/medical record should include:

- diagnosis and procedure,
- care or treatment provided
- any proposed recurrent care i.e., follow up appointments

These patients may receive other documentation, for example a transfer letter.

Mechanisms should be developed to provide timely information to GPs for exception-type patients to inform of procedures, findings, aftercare and follow-up; especially for investigative procedures such as endoscopy and angiography.

Refer to [NMHS Discharge Summary policy](#) for further information.

## 5. Rehabilitation and Other Discharge Services

Rehabilitation and other discharge service providers should be considered in planning discharge. Referrals to services should be initiated as early as possible to minimise discharge delays:

Common services include (see links for eligibility criteria and referral process)

- [Homelink](#)
- [Home Care Package](#) (contact Social Work ext. 74666)
- Rehabilitation in the Home (RITH) ([health.wa.gov.au](http://health.wa.gov.au))
- [Osborne Park Hospital](#) (OPH)
- [Fiona Stanley Hospital](#) (Ward A – Neurology, Ward B – Acute Brain Injury, Ward 1A – Spinal, Ward 2 A – Amputee, Trauma and Multidiagnostic)
- [Silver Chain](#)
- [Transitional Care Program \(TCP\)](#) (contact Social Work ext. 74666)
- [Rehabilitation and transitional care options for younger people with disabilities](#) (contact Social Work ext. 74666)
- [South Perth](#)
- Secondary Metropolitan Hospitals
- Private Hospitals/Regional/Rural Hospitals

## 6. Discharge Time

Standard discharge time is prior to 1000 hours, particularly when utilising the Transit ward. Optimal discharge planning by the multidisciplinary team should facilitate the patient being safely discharged from the ward on or before this time.

Patients and their families/carers should be informed of the discharge time to ensure transport and home arrangements are available and prepared.

If required, medical review must occur prior to this time OR the responsibility for discharge must be delegated to another staff member. Medical review can occur in the Transit ward.

## **7. Transit ward (SCGH only)**

All patients who have been discharged by the medical team and who meet Transit ward admission criteria must be sent to the Transit ward.

Patients can be moved to the transit ward to wait for:

- Transport
- Equipment
- Discharge summary
- Medications
- Test results

### **7.1 Overnight beds**

The eleven overnight beds (one single room for micro alerts) are to be used for the following patients:

- Confirmed for discharge the next day.
- Confirmed for transfer to another hospital or health service site the next day.
- Confirmed for discharge from the Emergency Department (ED) and are awaiting transport
- Transfers from ED waiting an admission for ward bed
- One single room is available for patients with:
  - Micro alert V, F, G, H, J and K
  - COVID 19 positive
  - Influenza-like illness (ILI)
  - Shingles

### **Exclusion Criteria**

- Unstable patients who meet Medical Emergency Team (MET) criteria
- Aggressive behaviours
- Dementia, delirium, or wandering behaviours.

### **7.2 Items to be sent with patient**

To transfer a patient to the Transit ward the following items are to be sent with the patient:

- A Discharge Envelope with the checklist commenced on the ward detailing the care still outstanding
- A completed Adult Transfer Form MR141.01 for patients transferring to another site
- For patients transferring to another facility, transport should be booked by the home ward the day of transfer to the DC ward
- Patient medical record containing all charts including the medication chart
- Patient to have an ID band insitu
- Discharge letter must have commenced by the medical staff
- Discharge medications should be documented and sent to pharmacy
- Patient's belongings and own medications
- Specialised equipment such as Zimmer, wheelchair •Medications such as IVabs, insulin

### **NOTE:**



Any modifications on the SCGOPHCG Observation Response Chart (ORC) should be reviewed prior to transfer and valid for up to 72 hours as per the procedure for recognising and responding to acute deterioration.

### 7.3 Identifying and booking patients in for overnight admission

- **Step One:** Ward Shift Coordinator to identify patients that meet the admission criteria.
- **Step Two:** contact the Bed Manager on ext 72395 to confirm acceptance of patient for transfer.
- **Step Three:** Notify the Transit ward on 77592 of patient for transfer.
- **Step Four:** Ward Shift Coordinator will use EBM to log patient for transfer.
- **Step Five:** Transit ward will confirm transfer on EBM.
- **Step Six:** Ward Staff (nursing or clerical) to log patient transfer onto CARPS.
- **Step Seven:** Ward nursing staff to escort the patient to the Transit ward and provide a clinical handover to the nursing staff.
- **Step Eight:** Once patient arrives, the Transit ward clerk transfers patients into the Transit ward and admits and discharges patients from SCGH using WebPAS.
- If unsure if your patient meets the admission criteria, please contact the Clinical Nurse Manager on ext. 75017, in hours. or the Bed Manager on ext. 72395, out of hours

### 7.4 Medical and Nursing Governance

- The Transit ward nursing team is responsible for administering general nursing care, including IMC, TOV, dressings etc.
- Patients will remain under the care of their admitting medical team whilst in the Transit ward.
- If the patient requires medical review the C Block CAT team will be contacted.
- For any urgent medical assistance, a code blue will be called by dialling 55.
- If staff require assistance with a patient in the Transit ward, the C Block After Hours CNS is to be contacted on ext. 75015.

## 8. Nurse Manager Discharge Coordination

The Patient Flow Unit provides a SCGH Discharge Coordinator team to expedite the discharge of patients with complex discharge needs.

Patients requiring discharge coordinators are identified by:

- Shift Coordinator
- Clinical Nurse Specialist
- Member of the multidisciplinary team

The criteria for identifying complex patients include:

- Multiple co-morbidities
- Involvement of 3 or more clinical specialties
- Homeless or other significant social barriers to discharge
- Guardianship application
- Frequent hospital presentations
- Discharge destination is within WACHS region, interstate or overseas with requirement for flights or significant coordination between sites
- Any other factor likely to result in the patient significantly exceeding the EDD

Discharge coordinators facilitate timely discharge of patients with complex discharge needs by:

- Developing management plans for individual patients
- Coordinating meetings with patient's family and/or carers
- Initiating referrals to inpatient, outpatient and community services
- Attending specialty team meetings and/or Journey Board meetings
- Coordinating transport arrangements for discharge

## 9. Journey Board Meetings

Journey board meetings should be:

- Concise
- Ward-based
- Multi-disciplinary discharge planning discussions.

Journey board meetings should focus on patients whose discharge is anticipated in the next 24-72 hours.

It is essential that the patient and next of kin/carers (where appropriate), be informed of any change to their care plan, including any revision to the EDD following a journey board meeting.

## 10. Long Stay Patients

Long stay patients are to be reviewed at the Long Stay Patient Committee meeting weekly and escalated via appropriate pathways as required.

## Compliance, monitoring and evaluation

Compliance with this policy will be measured against KPIs as determined by the SCGOPHCG Executive Committee and reported to relevant staff members for analysis and action.

100% of discharge summaries will be completed on the day of discharge or transfer to another health care facility

Target discharge numbers, based on admission numbers per ward, will be planned for on a 7/7 basis and EDD planning must follow service and admission rate requirements. EDD and discharge target breaches will be reported back to wards on a regular basis.

## Authorisation & Version Control

|                               |                                                                                     |                |            |                         |
|-------------------------------|-------------------------------------------------------------------------------------|----------------|------------|-------------------------|
| Policy Sponsor                | Area Director Nursing Services                                                      |                |            |                         |
| Policy Contact                | Director of Nursing, Patient Flow                                                   |                |            |                         |
| Date First Issued:            | 10/04/2023                                                                          | Last Reviewed: | 18/11/2024 | Review Date: 10/04/2026 |
| Version No.                   | 1<br>SCGOPHCG                                                                       | Reference #:   | HP214      |                         |
| Approved by:                  | Area Director Nursing Services                                                      |                |            | Date: 10/04/2023        |
| Endorsed by:                  | SCGOPHCG Policy and Guideline Committee                                             |                |            | Date: 10/04/2023        |
| Standards Applicable          |  |                |            |                         |
| Aboriginal Health ISD Number: | 2189                                                                                |                |            |                         |

For full details regarding, development, review, consultation, approval, endorsement - contact the Policy Officer for the submission form